

The one-afternoon underpayment check

Six steps to find out whether your insurers pay what your contracts say, using records you already have.

The six steps

From picking the insurer to the one number that matters. Page 2.

What isn't an underpayment

The lawful reductions to rule out before you count anything. Page 4.

What's still recoverable

Florida windows by insurer and the 12-month statutory floor. Page 6.

Where each number lives

The 835 fields and the paper EOB columns, side by side. Page 3.

The worksheet

A worked sample, then a blank sheet to print. Pages 4 and 5.

The appeal letter

One page, fill in the blanks, file by pattern. Page 7.

YOU NEED

Remittances for one insurer, 835 files or EOBs

AND

That insurer's fee schedule from your contract

TIME

An afternoon, once

Six steps

An underpaid claim doesn't look underpaid. The payment posts, the balance goes to zero, and the claim files itself under finished. The only way to know is to compare what each insurer allowed against what your contract says it owed. Here is that comparison, at a size one person can run in an afternoon.

1 Pick your highest-volume commercial insurer

Whatever pattern exists will be loudest where the claim count is highest. Start with one insurer and one fee schedule. Add the next insurer only after the first one is done.

2 Pull 20 to 30 zero-balance claims for your five most-billed codes

Zero-balance on purpose: you are auditing the claims everyone considers finished. Take recent ones first, paid in the last three months, because they are still inside every dispute window on page 6.

3 Write down three numbers per line

What you billed, what the insurer **allowed**, and what the contract's fee schedule says the allowed should be. The allowed-versus-contract comparison is the whole test. Ignore the paid amount for this step: paid moves with the patient's deductible and coinsurance, allowed doesn't. If your contract pays a percentage of a published schedule, compute the rate from that schedule for the right year and locality.

4 Check the code they paid

If the remittance pays a lower-valued code than the one you billed, that line was downcoded, even if the arithmetic on the paid code looks right. Three signatures: 99214 paid as 99213 or 99215 as 99214; adjustment reason 150, "information submitted does not support this level of service"; an allowed amount that lands exactly on the lower code's rate.

5 Ignore scattered pennies. Look for repetition.

A few tiny, random variances are noise: rounding, posting quirks, secondary-payer artifacts. What matters is the same code, the same insurer, the same gap or the same downcode, again and again. Underpayment that matters is a rule firing the same way on every claim that fits it. Rule out the lawful reductions on page 4 first.

6 Multiply by a year

Average gap per line, times the share of lines that were short, times how many times a year you bill that code to that insurer. That number decides whether this matters. A few dollars a line is a rounding error once. Across a year of volume it usually isn't.

Annual gap = average shortfall per short line × share of lines short × annual volume of that code with that insurer

Where each number lives

The 835 is the electronic remittance advice (ASC X12 005010X221A1): how each insurer paid, reduced, or denied every line. Posting software reads what it needs to close a balance and ignores the rest, which is where underpayment sits. Paper EOBs and portal PDFs print the same facts in columns.

WHAT YOU NEED	835 SEGMENT AND FIELD	ON A PAPER EOB	WHY IT MATTERS
Your claim number	CLP01, patient control number	Patient account or claim number	Ties the line back to your system
Insurer's claim number	CLP07, payer claim control number	Claim or ICN number	Goes on every reconsideration
Claim status	CLP02	Status or "processed as"	1 to 3 processed, 4 denied, 22 reversal
Date of service	DTM*472 at the service line	Service dates	Needed for the deadline math
Code paid	SVC01	Procedure code column	What the insurer adjudicated
Code you submitted	SVC06, present only when it differs from SVC01	Sometimes shown as "billed code"; often missing	If it differs from SVC01: downcode or recode. Posting usually ignores it
Billed charge	SVC02	Billed or charges	Confirms the line only
Allowed amount	AMT*B6	Allowed or eligible	The insurer's version of your contract rate. The number to test
Paid amount	SVC03	Paid or plan payment	Moves with deductible and coinsurance
Adjustments	CAS: group code, reason code, amount	Adjustment or remark columns	CO yours, PR the patient's, OA other. 45 fee schedule, 97 bundling, 150 level
Remark codes	LQ	Footnote codes	Often the only place the real reason appears
Provider-level adjustments	PLB	"Provider adjustments" at the end	Recoupments and offsets outside any claim line

Where to get the files

- **Your clearinghouse.** Every 835 it delivered is downloadable, usually for at least a year. Ask for the raw files, not the formatted report.
- **Insurer portals.** Availability and the insurers' own portals let you pull remittances by check date, including insurers that bypass your clearinghouse.
- **Your billing company.** If they post for you, the files pass through them. They're your records; ask for the archive.

Keep at least twelve months. Florida's underpayment window on state-regulated plans runs twelve months from the payment date, and a file you don't have is a claim you can't make.

What isn't an underpayment

Most adjustments are correct. Disputing correct ones burns goodwill with the insurer and buries your real claims in noise. Rule these out before you count anything.

- **Patient responsibility.** Deductible, coinsurance, and copay sit under group code PR. They lower the paid amount, not the allowed amount, which is why step 3 tests allowed.
- **The routine contractual write-off.** CO-45 is the gap between your charge and your contract. When the allowed ties to the fee schedule, it's the deal you signed. Post it and move on. When the allowed is below the fee schedule, the shortfall is hiding inside that same CO-45.
- **A legitimate multiple-procedure reduction.** Second and later procedures commonly pay at 50 percent. The ranking should run by fee-schedule value, highest first. A reduction applied to your highest-valued procedure is a shortfall; the correct one isn't.
- **Medicare sequestration.** Two percent of the Medicare payment amount, not of the allowed amount, under its own adjustment code. Recompute it on that basis before questioning it.

A worked sample

Sample, synthetic figures. One commercial insurer, an orthopedic practice, 30 lines checked.

CODE BILLED	CODE PAID	ALLOWED	CONTRACT	GAP	ADJUSTMENT	READ
99214	99214	\$142.00	\$142.00	\$0.00	CO-45	Correct
99214	99213	\$98.00	\$142.00	\$44.00	CO-150	Downcode, 9 of 30 lines
20610	20610	\$88.40	\$96.10	\$7.70	CO-45	Rate shortfall, 12 of 12 lines
73030	73030	\$31.20	\$31.20	\$0.00	CO-45	Correct
99203	99203	\$121.00	\$121.00	\$0.00	PR-3	Copay, not a gap

Annualized. 99214 downcodes: $\$44.00 \times 30$ percent of lines $\times 1,400$ a year with this insurer = **\$18,480**.
20610 shortfall: $\$7.70 \times$ every line $\times 600$ a year = **\$4,620**. One insurer, two codes, **\$23,100 a year**.
Neither pattern is visible on a posted, zero-balance claim.

The downcode is a documentation dispute: the reconsideration attaches the note and states whether the level rests on medical decision making or total time on the date of service. The rate shortfall is arithmetic: contract page and recompute.

[illegible]

CODE	LINES CHECKED	LINES SHORT	AVG. GAP PER SHORT LINE	ANNUAL VOLUME, THIS INSURER	ANNUAL GAP

The one-afternoon underpayment check · 5

What's still recoverable

Every finding has a clock. Two run at once: the insurer's contract and manual, and, for underpayments on state-regulated plans, Florida's statute. Work the soonest-expiring dollars first.

Contract and manual windows, contracted providers in Florida

INSURER	FIRST STEP	NEXT STEP
Florida Blue	Reconsideration within 1 year of the remittance date; required before appeal	Administrative appeal inside the same year
UnitedHealthcare	12 months from the EOB or remittance date	Reconsideration and appeal must both fit inside those 12 months
Aetna	Reconsideration within 180 days of the initial decision; underpayments go here	Appeal within 60 days of the reconsideration decision; one level
Cigna	180 days from the initial payment or denial	One internal level, then arbitration per contract. Make the first one complete
Humana	About 120 days from the remittance; a working figure, confirm in your contract	60 days from the first-level decision
Medicare, traditional	Redetermination within 120 days of receiving the remittance	Reopening: 1 year for any reason, 4 years for good cause

Your contract can shorten these, and the deadline printed on the EOB or remittance controls over any table. Denials run on these clocks only; the statute below is for claims paid short.

The Florida floor for underpayments

12 months from the payment date. A claim for underpayment by a physician, osteopath, chiropractor, podiatrist, or dentist must be submitted within 12 months after the insurer's payment. Section 627.6131(19), Florida Statutes; for HMOs, section 641.3155(17).

It can't be contracted away. The section may not be "waived, voided, or nullified by contract," section 627.6131(10). A 90-day dispute clause doesn't erase the statutory window on covered business.

12 percent simple interest on overdue payments, section 627.6131(7); for HMOs, section 641.3155(6). State it in the letter.

Where it doesn't apply. Self-funded employer plans under ERISA, even when a familiar Florida carrier administers them and the card looks the same. Medicare Advantage. Federal employee plans. Workers' comp, which runs on its own 45-day petition clock from receipt of the EOBR. You usually can't tell self-funded from the card: check the group number or the benefits documents, claim by claim.

Triage rule. Underpayments on fully insured business stay alive 12 months everywhere. Traditional Medicare shortfalls, a year through reopening. Denials older than about four months are already closing at Humana, Aetna, and Cigna.

The one-page appeal letter

Not a medical argument. A recompute: here is the contract rate, here is what you allowed, here is the difference, please reprocess. No adjectives, no history of the relationship.

[Practice letterhead] [Date]

[Insurer], provider disputes and reconsiderations, [address, fax, or portal reference from the provider manual]

Re: Request for reprocessing, underpayment against contracted rate

Tax ID [] · NPI [] · [n] claims, list attached

The [n] claims on the attached list, dates of service [from] to [to], were allowed below the contracted rates in our agreement effective [date], [exhibit or section]. For example, claim [#], date of service [date], CPT [code], was allowed at \$ []; the contracted rate is \$ [].

For a downcode, add: The level billed is supported by [medical decision making: elements] or [total time of [] minutes on the date of the encounter]. The note is attached.

Please reprocess each claim at the contracted rate and remit the difference of \$ [total], with 12 percent simple interest under section 627.6131(7), Florida Statutes.

Attached: 1. remittance lines; 2. contract page, rate highlighted; 3. claim list with the recompute per line; 4. proof of timely filing, if the insurer's history suggests it will argue the point.

[Name, title, phone, email]

The interest clause belongs only on Florida fully insured plans; for an HMO cite section 641.3155(6). Leave it out for self-funded plans, Medicare Advantage, and federal employee plans.

Four rules that decide whether it pays

1. **Reconsideration first.** It's faster, it's usually handled by claims staff rather than medical reviewers, and it keeps the formal appeal in reserve.
2. **Use the portal or the address the manual names for payment disputes.** A portal timestamps receipt. A dispute sent to the medical appeals queue can sit for months.
3. **File by pattern, not by claim.** One underpaid line isn't worth an hour. Every line from that insurer, that code, that quarter, in one claim list with one contract page, usually is.
4. **If they say your rate is different,** you've found a contract-loading problem. Fixing it in their system stops the shortfall on every future claim, which is often worth more than the appeal.

If the afternoon isn't available, Clearia's diagnostic runs this check on every insurer and every line of the last twelve months, at no cost. We're paid only from what we recover. cleariagroup.com/the-diagnostic

General information, not legal advice. Statutes, rules, and insurer policies change; verify the current text and your own contracts before acting on any deadline or rate cited here. Figures on page 4 are synthetic.